

Medical Device Malfunction, Recall, and FDA Reporting

Policy Number	POL-SAF-073	Owner / Department	Clinical Engineering / Risk Management
Effective Date	2024-02-01	Version	3.0
Last Revised	2026-05-04	Next Review	2028-05-04
Approved By	Environment of Care Committee	Applies To	All staff and providers who use, maintain, or manage medical devices, and Clinical Engineering, Supply Chain, and Risk Management

1. Purpose and Scope

This policy defines how Riverside Regional Medical Center, as a **device user facility**, identifies, sequesters, investigates, and reports medical-device malfunctions, serious injuries, and deaths, and how it manages recalls. It implements the Safe Medical Devices Act and FDA Medical Device Reporting requirements at **21 CFR Part 803**.

2. Immediate Actions and Sequestration

1. Ensure patient safety first; if a device is implicated in harm, remove it from service.
2. **Sequester** the device and all associated accessories, packaging, disposables, and settings — do not alter, clean, reset, or discard them.
3. Tag the device “Do Not Use — Quarantine for Investigation” and notify Clinical Engineering.
4. Record device make, model, serial/lot number, and settings at the time of the event.
5. Notify Risk Management, which determines FDA/manufacture reportability.

IMPORTANT — Do not return, repair, discard, or reset a device involved in a death or serious injury. Sequester it exactly as found — it is evidence, and FDA/manufacture reporting deadlines begin the moment the facility becomes aware.

3. FDA Reporting Timeframes

Reports are submitted **as soon as practicable** and no later than the deadlines below, measured from when the facility becomes aware of the event (21 CFR 803.30, 803.32). Deadlines are counted in **work days**.

Event	Report to	Deadline
Device-related death	FDA (MedWatch 3500A) and manufacturer, if known	Within 10 work days
Device-related serious injury	Manufacturer; FDA if manufacturer unknown	Within 10 work days

Event	Report to	Deadline
Annual summary of device reports	FDA	By January 1 each year

A device **malfunction** that did not cause death or serious injury but could if it recurred is investigated and may be reported to the manufacturer; the manufacturer, not the user facility, carries the 30-day MDR obligation to FDA.

4. Recall Management

- Clinical Engineering and Supply Chain triage FDA and manufacturer recall/safety notices on receipt and classify urgency (Class I, II, III).
- Affected devices and lots are located, removed from service, quarantined, and reconciled against inventory.
- Clinical units are notified of substitutes and interim workarounds; completion is documented and tracked to closure.

5. Severity & Reporting

Classify and notify per **POL-RM-013**. A device-related death or a malfunction causing serious patient harm is a **SEV-1**; a malfunction with major impact but no serious injury is a **SEV-2**; a minor malfunction or a Near Miss is a **SEV-3 / Near Miss**. Internal severity notification is independent of — and does not replace — the FDA/manufacturer MDR deadlines above.

Severity	Notify	Deadline
SEV-1 (catastrophic / sentinel)	Clinical Engineering, Risk Management, administrator on call	Within 1 hour
SEV-2 (major)	Clinical Engineering and department manager	Within 4 hours
SEV-3 / Near Miss (minor)	Clinical Engineering via electronic event report	Within 24 hours

6. References

- Safe Medical Devices Act of 1990.
- 21 CFR Part 803 — Medical Device Reporting (user facility 10-work-day death and serious-injury reports; manufacturer 30-day reports).
- POL-RM-013 — Incident Reporting and Event Severity Classification.
- POL-SAF-071 — Magnetic Resonance Imaging (MRI) Safety.